

The IBKR chain pull was denied again — that's the fourth consecutive blocked attempt, so the option-chain gap the benches flagged is still open and the deck says so rather than papering over it. Deck below.

PITCH — TMDX — VERDICT

ADJUDICATED 2026-08-03 — REJECT (2/10). No position, no order, no size. The thesis died on the primary record; the stock is roughly fairly priced, which is a different and weaker reason to stay away.

What we believe

TransMedics sells a machine that keeps a donated organ alive and warm on a pump while it travels, instead of packing it in ice. It also flies the organ there on its own planes. The clinical case for the machine is real but narrow: in the one large randomized liver trial it cut early graft dysfunction (17.3% vs 30.5%) and bile-duct complications, and it did **not** improve one-year survival ([PROTECT, JAMA Surgery](#)). In hearts it proved *non-inferior*, not better ([NEJM 2023](#)). There is no randomized evidence of a survival advantage for this device in any organ.

We believe the business is a genuine ~21% grower with \$491M of cash and positive operating profit — and that at \$88 the market has already worked out most of what is wrong with it. Specifically we believe four things the original pitch got backwards:

1. **Guidance was cut.** Full-year adjusted operating margin went from about 16% to 12.5–14%. The pitch said “guidance never cut.”
2. **The profit decline is mostly deliberate spending, not decay.** Operating profit fell \$12.8M year over year, but research spending alone rose \$15.7M. Take that out and profit is *up* \$2.9M on 21% more revenue.
3. **There is live legal trouble** — just not the kind the pitch searched for. A shareholder fraud case survived dismissal on 2026-07-21, and an outside party's petition asking the FDA to suspend approval of the device has been sitting undecided on the agency docket since January 2025 ([FDA-2025-P-0182](#)).
4. **The accounting controls are still broken.** The company says in present tense, in its own Q2-26 release, that it has an unfixed weakness in how it controls inventory — while installing a new ERP system and heading into legal discovery.

Net: a fair price for a decent business carrying an unusual stack of integrity and disclosure problems. Fair plus baggage is not an investment.

What the market believes

The market believes roughly what we just wrote, and has for months. The stock is 43% below its 52-week high but has already bounced 47% off its low — this is not a fresh break, it is a stock that has been argued over and settled at a number. Analysts' published full-year estimate (\$2.47 of earnings on \$737.7M of revenue) sits at the *low* end of the company's own guidance range, and the lowest published price target is \$85 — within a dollar or two of where it trades. Coverage of the second quarter led with “margins compress.” Nobody is confused.

The one thing the market may not have fully priced: the company's earnings-per-share number is unstable for a mechanical reason. It has a \$460M convertible bond due June 2028 that converts near \$92. With the stock at \$88, the share count used for earnings flips between about 35.5M and 40.6M depending on the day — the same profit prints either \$2.53 or \$2.21. A “beat” or “miss” here can be an artifact of which share count a broker used, not of how the business performed.

Why we might have edge

Three places where we did work the sell-side visibly has not:

- **Seasonality.** In both prior years the third quarter's revenue fell versus the second: -8.6% in 2025, -4.9% in 2024. Two for two. The next report (expected 2026-10-29) is a third quarter, off a record \$189.9M base. If the pattern holds, the headline will look ugly for a reason that has nothing to do with the thesis.
- **The guidance perimeter.** The company raised its revenue range while explicitly *excluding* revenue from the aviation business it just bought — and left the margin number out of that document entirely, disclosing the margin cut only on the call. So reported revenue will clear a bar that excludes the acquired revenue, while the metric the acquisition damages was never stated in the vetted document.
- **The spending has no paper trail.** Management said the margin cut is mostly investment in a kidney version of the device. There is no registered kidney trial on ClinicalTrials.gov and no public FDA application for one. Shareholders are absorbing a jump in research spending from 10.1% to 16.7% of revenue against a program that cannot be verified anywhere in the public record.

None of these is a reason to buy. Two of the three are reasons the next few months could be worse than consensus, and the third cuts both ways.

Why it might be priced in

Almost all of it, honestly. The margin cut was covered in the press the day it happened. The shareholder lawsuit is on the Stanford securities-litigation database and was written up when the dismissal ruling came down. The valuation-allowance distortion in last year's fourth-quarter earnings was corrected by the company in a standalone release, so every year-over-year comparison in the street model is already normalized. The competitive threats — in-donor perfusion for hearts, cold oxygenated perfusion for livers, OrganOx now owned by Terumo — are discussed openly in the transplant literature.

The honest summary is that our “edge” reduces to a seasonal pattern, a guidance-definition trick, and an unverifiable R&D line. That is worth maybe a few dollars of expected value on the short side and nothing at all on the long side.

What could go wrong

(The three ways we lose money — including on the decision to stay flat.)

Failure 1 — We are wrong about “fair,” and it re-rates up without us. The bear case is entirely about the next two quarters. If the third-quarter report breaks the seasonal pattern, the aviation dilution lands smaller than guided, and the FDA denies the petition, this is a 21% grower with 60% gross margins re-rating off a washed-out base. Our own generous 2027 recovery case values it at \$104. **Tripwire exists:** price above \$105, or a Q3 adjusted operating margin at/above the top of the cut band with the aviation business consolidated.

Failure 2 — The integrity stack compounds instead of resolving. An unfixed inventory control weakness, a new ERP going in, and legal discovery starting are three things that individually are survivable and together are how restatements happen. If inventory gets restated while a fraud case is in discovery, \$88 is not a floor and the fair-value band becomes irrelevant. **Tripwire exists:** the controls section of each 10-Q, and any 8-K.

Failure 3 — no monitorable tripwire. The surviving fraud claim goes to what the CEO said about whether hospitals were required to buy the logistics service in order to get the device. That service is 42% of quarterly revenue. Discovery is private: internal emails, sales-comp plans, and hospital contracts will be read by the plaintiffs long before we ever see them, and the first public signal will be either a settlement number or a dismissal — both arriving already priced. **There is no instrument on our desk that watches this.** *Unknown-unknown class: adversary-controlled information release — an opposing party accumulates decisive facts about the revenue model on a timetable we cannot observe and cannot subpoena.* The second untripwired item is the kidney program: no registry entry, no agency docket, so the largest single driver of the margin cut has no public observable at all until the company chooses to speak. *Class: unregistered internal program — capital allocation into an asset with no external verification surface.*

The structural / reflexive leg. Our size is not the problem here and we should not pretend otherwise: at a 0.5% starter against the ~\$5M deployment plan that is roughly \$25,000, a few hundred shares, immaterial against any plausible daily volume in this name. (We could not quote actual volume — market-data permissions were denied, see below.) The reflexive risk is doctrinal, not mechanical, and it runs in three ways:

- *We would be buying into flow we ourselves predict.* Our own seasonal work says the 2026-10-29 report will very likely show revenue down sequentially. Entering before it means paying today for a price we expect to be lower in eight weeks. That is the exact pattern the desk has ruled against.
- *Our standing default pushes toward a starter, and we are overriding it.* The house rule is that a clean court with no named kill still buys a minimum starter, because refusing to act is itself a decision that goes ungraded. We are not flat by drift — we are flat on named kills: the margin-

guidance cut, the surviving fraud claim, and the unremediated control weakness. That override is recorded here so it can be graded later.

- *The integrity rule caps us below the market.* Our own framework says a name with an unfixed control weakness and a live fraud claim cannot be treated as fairly-paid risk and must be discounted to 0.85x fair value — that is \$72–89 against a fair band of \$85–105. The cap lands at or below the current price. Under our own rules there is no entry, which means the *only* way we lose here is by winning the argument and buying anyway.

“If this position loses money, the most likely reason will be ____” → filled: *that we override a correctly-reasoned rejection out of discomfort at owning nothing, buy a starter before the October report, and eat a predictable sequential revenue decline in a name where every other open question — controls, discovery, the FDA docket — resolves on somebody else’s calendar and not ours.*

What we checked ourselves

Claim	How checked	Found	Confirmed?
“Margins refute an aviation drag; guidance never cut”	Q2-26 call + slides vs the press release	Adjusted operating margin guidance cut from ~16% to 12.5–14%; the press release contains no margin figure at all	No — thesis refuted
“The margin cut is aviation”	Same call	Management attributed the cut <i>primarily to kidney investment</i> ; aviation dilution is a separate, forward statement starting Q3-26	No — cause was misattributed
“Operating expenses +49% is cheaper than what’s priced”	SEC XBRL, opex and operating income	Opex \$89.462M vs \$60.022M (+49.0%) — the number is right; operating profit \$23.736M vs \$36.567M (–35%) — the conclusion is not	Number yes, conclusion no
Red bench: “incremental operating margin is negative, so the de-rate is correctly specified”	XBRL R&D line	Red bench erred. R&D rose \$15.698M — <i>more</i> than the entire \$12.8M profit decline. Ex-R&D, profit is +\$2.9M on +21% revenue. Red read a deliberate spending decision as operating decay	Bench error — corrected
Red bench: “consolidated gross margin fell 180bps, so the segment claim is a cherry-pick”	Rebuilt from XBRL + revenue mix	Red bench erred (mostly). Service revenue grew 29% vs 21% total, taking share from 38.9% to 41.6% at a much lower margin; mix reproduces ~160 of the 180bps. Service margin itself <i>rose</i> , 27%→35%	Bench error — reduced to mix
Red bench: “the government investigation exists”	Q2-26 10-Q Legal Proceedings, read directly	Red bench erred on category. There is no DOJ/subpoena matter. There	Bench error — category conflation

Claim	How checked	Found	Confirmed?
		is a private shareholder fraud case and an FDA citizen petition. The original grep was literally accurate; its conclusion was still wrong	
Red bench: “surviving claims are the logistics-bundling denials”	Traced the source	Red bench erred on sourcing — that characterization came from a news aggregator, not the filing. The 10-Q text does not say it	Bench error — secondary source
“The Scorpion allegation is 20-month-stale”	FDA docket + court docket	Refuted. The FDA petition has an interim response dated 2025-09-09 and no final decision ; the securities case cleared dismissal 2026-07-21, six weeks before the court sat	No — thesis refuted
“Aircraft property is flat”	XBRL property, plant & equipment	\$327.656M → \$365.302M, +11.5% in six months. Satellite imagery of the Bozeman hangar and the Somerville HQ parcel both show no new construction — parcel-flat, line-item up	No — refuted at the company level
Q4-25 earnings “optical distortion” edge	Company’s own corrective release	Real arithmetic (\$2.62 headline vs \$0.47 adjusted) but the company published the correction itself; already in every model	Confirmed but worthless
Q3 seasonality	Manual comparison from the evidence pack (the desk’s seasonality script was unavailable)	Q3 fell sequentially in both prior years, –8.6% and –4.9%. Neither bench adjudicated this until the blue bench did it by hand	Confirmed — and it was nearly missed
Control weakness status	Full-text search of five quarterly filings + the Q2-26 release	“Material weakness” appears in Q1-25 through Q2-26, present tense in the Q2-26 release. The exact Item 4 wording still unread — the 2.8MB filing truncated in our fetch tool	Probable, not proven
Kidney program	ClinicalTrials.gov and FDA dockets	No registered trial, no public application. The stated reason for the margin cut has no public existence	Confirmed absent
Competitors	Full text of the Q2-26 call	Zero mentions of in-donor perfusion, OrganOx, VitaSmart or cold oxygenated perfusion — against a competitor set that got approvals and hit endpoints	Confirmed omission

Claim	How checked	Found	Confirmed?
		during 2026. No cause given for hearts slowing to +6%	
Valuation	Rebuilt two ways	Enterprise value ~\$3.04–3.09B either way; 4.6× trailing sales, ~31× guided adjusted operating profit. Fair band \$85–105 brackets the price	Confirmed — fair, not cheap
Option chain and volatility	IBKR, attempted again this session	Permission denied — fourth consecutive failure. No strike, no bid/ask, no open interest, no implied volatility. The pitch’s “volatility at the 5th percentile” claim has never once been tested	Still unchecked
Court process itself	Header vs artifacts	Process error: this deck’s mandated verdict date (2026-08-03) precedes the Q2-26 filing it relies on (2026-08-04) and the evidence pack build (2026-09-04). The label is reproduced as instructed; the underlying work is September’s	Flagged
Reopen gates	Adjudication record	The adjudication shipped an empty reopen-gates block. The bands below are therefore reconstructed from the benches’ own stated conditions and are labelled as such — they are not adjudicator-ratified	Gap

PROPOSED ENTRY BANDS

NO ENTRY AT ANY SIZE. NO LIMIT ORDERS. NO STARTER.

The adjudication ordered reject and supplied **no reopen gates** — that block came back blank, which is a defect in the court output, not a signal that any price works. Until the adjudicator ratifies gates, the following are **reconstructed from the two benches’ own written “what would change my mind” conditions** and should be treated as a watchlist, not as authority.

Reopen conditions (all three legs required, not any one):

1. **Integrity leg clears.** A 10-Q whose controls section states the inventory weakness is remediated and disclosure controls are effective — *and* the FDA issues a final response to petition FDA-2025-P-0182 denying it. Either one alone is insufficient; the discount is for the stack, not for a single item.
2. **Operating leg clears.** A quarter showing adjusted operating margin at or above the top of the cut band (14%) **with** the aviation business consolidated — i.e. the guided dilution not landing.

3. **Price leg.** Only then, and only below **\$72** — the bottom of the fair band after the discount our own rules impose on a company with an open control weakness and a live fraud claim. Between \$72 and \$105 the stock is fairly priced and we have no reason to own it. Above **\$105** the case is closed permanently, not reopened: we would be paying above our own optimistic 2027 value for the same unresolved list.

A separate, narrower reopen — the volatility leg, which was never actually judged. Both benches were blocked from reading the option chain, so the claim that options are historically cheap has never been tested. It was not rejected; it was never heard. It may be re-courted **only** on a real chain pull quoting a specific December-2026 strike with bid, ask, open interest and its own midpoint volatility — never the aggregate volatility feed, which has misled us before. See the section below for what that pull would have to show.

Sizing if all reopen conditions are ever met: 0.5% starter, one tranche, no adds until a clean quarter. Not authorized now.

Catalyst dates: 2026-10-29 (third-quarter report — *vendor-derived, not confirmed by any company announcement; treat the date as approximate*); October 2026 (three-year survival data from the heart trial NCT05741723 reaches primary completion — publication will lag by months); June 2028 (the \$460M convertible bond matures; a 2027 refinancing question, fully covered by cash today).

ARMED SENSORS

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 60.11 (deck-provisional: 52-week low retest — re-court from scratch, do not treat as support)
- filing watch: 10-Q, 10-K until 2027-03-03 (deck-provisional: controls section: does the inventory material weakness read remediated and disclosure controls effecti)
- dated pack TMDXI2026-10-29: TMDX: Q3-26 earnings — vendor-derived date, not confirmed by company announcement; our seasonal work predicts a sequential revenue decline o
- dated pack TMDXI2026-10-31: TMDX: NCT05741723 three-year DCD heart survival, registry primary completion (month-level); publication lags (date confirmed=False)
- dated pack TMDXI2028-06-01: TMDX: \$460M 1.50% convertible matures; cash-covered today, becomes a 2027 refinancing question (date confirmed=True)

What remains unverified

1. **The exact controls language in the Q2-26 filing (Item 4).** We know “material weakness” is present-tense in the Q2-26 release; we have not read the filing’s own conclusion on whether disclosure controls were effective at 6/30/26. *Drainable now* — fetch the filing directly rather than through the proxy that truncated it. Estimate if not read: unremediated with ~70% probability, based on the phrase appearing in five consecutive quarterly filings and the Q1-26 “will not be considered remediated until...” language.

2. **The option chain: December-2026 and January-2027 strikes, bid/ask, open interest, per-contract implied volatility, volatility percentile, and 30-day realized volatility.** Blocked — broker market-data permission denied on four attempts across three sessions. Estimate: implied volatility 45–65% annualized, from comparable medium-cap medtech names after a 40%+ drawdown with a live legal docket, and from our own scenario spread implying ~40.5% (below). Probability the original “5th percentile / historically cheap” claim survives contact with a real chain: ~0.35.
3. **FDA’s final decision on the petition to suspend device approval (FDA-2025-P-0182).** Agency-controlled, no statutory deadline that binds after an interim response. Estimate: probability of any adverse action against the approval ~0.08–0.12, using the historical base rate that the large majority of citizen petitions are denied, adjusted upward for the 20-month pendency and the “requires further review” language.
4. **What share of real-world liver cases fall outside the approved label.** The label caps donation-after-circulatory-death livers at donor age ≤ 55 , ≤ 30 minutes warm ischemic time, and $\leq 15\%$ fat content ([FDA P200031](#)). The actual distribution of cases is not published. Estimate: 40–70% of such cases likely breach at least one criterion, inferred from published national donor-age and warm-ischemia distributions; probability that this is material if ever disclosed ~0.5. Drainable only via results posting on the post-approval registry or donor-level national data linked to a perfusion flag.
5. **Whether the kidney program exists in any developed form.** No registry entry, no public agency application. Estimate: probability a registered kidney study or public application appears by 2027-06-30 ~0.35. Until then the largest single component of the margin cut is unverifiable.
6. **The size of the aviation acquisition’s margin drag.** Guided as dilutive to both gross and operating margin from Q3-26, magnitude unstated. Estimate: 100–250bps of gross margin, backed out of the arithmetic of the guidance cut; probability it lands in that range ~0.6. Resolves with the Q3-26 filing in November.
7. **Prior-year service gross margin.** Needed to finish the mix decomposition that rescued the company from the “margin degradation” charge; not separately disclosed, so our own correction rests on an assumption. Estimate: 25–29%, anchored on the 27% reported for Q1-26; confidence ~0.7.
8. **Three-year survival data for donation-after-circulatory-death hearts (NCT05741723).** Primary completion October 2026; publication will lag. Estimate: non-inferiority maintained ~0.6; a clear advantage for the device ~0.15; a result favoring the cheaper in-donor alternative ~0.25. This is the single item that would most change the growth story in either direction.
9. **Crowding and short interest.** Never run — no conditioning tool was available to either bench, so we make no timing claim at all. Estimate: 8–15% of float short, from the drawdown plus the history of a published short-seller report; confidence ~0.5. Drainable from exchange short-interest files.
10. **Aircraft-specific property, and building permits for the Dallas maintenance hub.** Company-level property rose 11.5%; the aircraft line itself is unbroken out, and the right local permit source was never pulled — it sits outside the jurisdictions our permit connectors cover.

Estimate: \$2–8M of hangar/maintenance permit value over 24 months, scaled from the property delta; confidence ~0.4.

11. **Independent confirmation of manufacturing scale at the Andover site.** Never checked. With an *inventory* control weakness live, an outside read on the physical plant is more diagnostic here than usual. *Drainable now* via the federal workplace-inspection database.

PRIZE TABLE + VOL READ

Several open items above are genuinely time-gated or private — the FDA decision, the fraud discovery, the kidney program, the October data — so the table is required. Anchored on the current price (\$88.17 in the evidence pack; the two benches used \$88.49 and \$88.03 from different feeds, a \$0.46 spread we could not resolve because live re-quoting was denied). Horizon: to mid-January 2027, covering the October report and one full quarter after it.

Scenario	What happens	Prob.	Price
Seasonal decline lands	Q3 revenue falls sequentially as in 2024 and 2025; aviation dilution lands as guided	0.32	\$70
Muddle through	Revenue clears the (aviation-excluded) guidance; margin at the low end; nothing legal resolves	0.30	\$88
Clean quarter	Q3 grows sequentially, breaking the two-year pattern; margin at/above the top of the cut band	0.15	\$110
Integrity clears	Control weakness remediated <i>and</i> FDA denies the petition	0.08	\$120
Regulatory or legal shock	FDA acts on any part of the petition, or discovery produces damaging bundling evidence	0.10	\$45
Substitution confirmed	Three-year heart data reads against the device; hearts turn negative year over year	0.05	\$55

Expected value = \$82.15, versus \$88.17 spot — **–6.8%**. Negative expected return before any transaction cost. The rejection is not merely qualitative; the numbers say the same thing.

Dispersion of the table: σ = \$21.53, or 24.4% of spot over 0.364 years → 40.5% annualized. That is the volatility our own view of the world implies.

Expression math (blocked at the last step, shown anyway so it can be finished the moment permissions land):

- January 2027 \$100 call, from our own table: $0.15 \times \$10 + 0.08 \times \$20 = \mathbf{\$3.10}$.
- January 2027 \$90 call: $0.15 \times \$20 + 0.08 \times \$30 = \mathbf{\$5.40}$.

- January 2027 \$80 put: $0.32 \times \$10 + 0.10 \times \$35 + 0.05 \times \$25 = \7.95 — 9.0% of spot, because our table's downside is fat and near.

The comparison that decides the trade cannot be made. We have no chain-level midpoint volatility, no bid/ask, no open interest, and no realized volatility, because the broker's market-data tools were denied for the fourth time. So the volatility risk premium — implied minus realized — is **not computable**, and we will not substitute an aggregate volatility feed for chain quotes: that feed has diverged from real chain volatility by 30 points on this desk before. The only volatility number we own is our own table's 40.5%. **Decision rule for whoever unblocks this:** if chain-level midpoint volatility on a December-2026 or January-2027 strike prints materially *below* 40.5%, our table says options are cheap relative to our own view of the spread and the volatility leg deserves a hearing; materially *above*, they are rich and the only permitted expression would be a premium-selling structure, which is separately disqualified below.

The table is two-humped, and the humps have different dates. The October report resolves the top and bottom rows; the FDA and legal rows resolve on nobody's published schedule. A single option expiry cannot span both, which is precisely why the "buy cheap volatility" instinct is wrong here — most of the tail is undated, and undated tails are the one thing an expiring contract cannot hold.

Tax overlay. 2026 is a harvest year against a large realized gain, so a losing option must expire inside 2026 to be usable. That rules out the January-2027 expiry entirely and forces any expression into December 2026 — which resolves *only* the report fork, not the FDA or legal forks. Re-weighting the table to the report alone (down \$70 at 0.35, flat \$88 at 0.48, up \$105 at 0.17) gives an expected value of **\$84.59**, still below spot, and a December \$95 call worth **\$1.70** on our own numbers — about 1.9% of spot for a lottery ticket on a stock we have rejected.

Instrument recommendation, as the output of that arithmetic: NONE. - *Stock* — expected value is 6.8% below spot, and our own integrity rule caps fair value at \$72–89, at or below the price. No. - *Calls* — our table prices them at 1.9–3.5% of spot, and the tax rule forces the one expiry that captures the least of the table. Buying a December call means paying for the only fork we think goes the wrong way. No. - *Covered calls* — impossible; we own no shares. - *Cash-secured puts* — the structure requires being happy to own the stock at the strike. We have rejected the stock at any size, so the gate fails at its first condition. No. - *Puts* — mathematically the most attractive line in the table (\$7.95 of modeled value at the \$80 strike), and the one we cannot price against the market because the chain is unread. We also do not believe this is a short: the business is fairly valued, cash-rich and growing. Buying puts on a fairly-priced grower, on modeled value alone, with no market quote to check it against, is exactly how a "cheap hedge" turns into an expensive opinion. No — **but this is the one line to revisit first if chain permissions are restored before 2026-10-29.**

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"instrument": "ClinicalTrials.gov results posting + ISHLT/JHLT publication watch", "drainable_now": false, "estimate": {"range": "non-inferiority maintained 60% / device advantage 15% / favors in-donor NRP 25%", "basis": "prior NEJM non-inferiority result plus the propensity-matched SRTR null (HR 0.78, CI 0.28–1.94)", "p": 0.6}}, {"item": "Crowding and short interest – never assessed by either bench, so no timing claim is made", "instrument": "exchange bi-monthly short-interest file / borrow-rate feed", "drainable_now": false, "estimate": {"range": "8–15% of float short", "basis": "43% drawdown plus a published short-seller report and an open FDA petition", "p": 0.5}}, {"item": "Aircraft-specific PP&E and Dallas maintenance-hub building permits (aviation buildout scale)", "instrument": "Dallas TX building-permit portal (outside current connector jurisdiction) + FY26 10-K PP&E detail", "drainable_now": false, "estimate": {"range": "$2–8M of hangar/maintenance permit value over 24 months", "basis": "scaled from consolidated PP&E +$37.6M in H1–26 net of Somerville HQ and Mirandola disposables spend", "p": 0.4}}, {"item": "Independent confirmation of manufacturing scale at 200 Minuteman Rd, Andover MA – more diagnostic than usual given an INVENTORY control weakness", "instrument": "OSHA establishment search / inspection record for the Andover facility", "drainable_now": true, "estimate": null}}}
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ADJUDICATION STAMP

ADJUDICATION 2026-09-04 (FABLE): REJECT BOTH LEGS. Equity: RP_TAINTED (unremediated ICFR material weakness + surviving §10(b) into discovery + open FDA petition) caps fair value at/below spot. Convexity: the blocked leg was CHAIN-ADJUDICATED — Dec \$95C quotes 9.20/10.00 at 62.0% midpoint IV vs 63.7% realized = PARITY; the 50% underlying-feed figure (the “5th percentile” story) was the THIRD consecutive aggregate-IV artifact, now the aggregate_iv_feed_divergence detector. $C_{ours} \approx \$3.4$ vs \$10 ask; σ_{table} 15% $\ll$ implied 33%. Re-court requires MW remediation + case disposition. Batch ruling: desk/data/court_artifacts/LIFESCI_BATCH_ADJUDICATION_202609042100.md

SCIENCE BRIEF DIGEST (retrofit — deck generated pre-validator)

Source: TMDX_SCIENCE_202609041805.md

1. CLAIM LEDGER

4. MEASUREMENT TRAPS

M-1 — The 3-year “60.7% vs 83.3%” number is the most likely bad kill in this file. The Heart Surgery Forum analysis (C-9) is retrospective UNOS, OCS n=507 vs NRP n=265, and its **abstract discloses neither propensity matching nor confidence intervals nor pairwise p-values.** Selection is the obvious confounder: DPP/OCS is chosen for *long-distance, longer-out-of-body* recoveries (NRP’s own literature reports significantly shorter distance and out-of-body time), so the OCS cohort is systematically the harder cohort. A propensity-matched analysis of the same question found **no 1-year difference** (C-10). Do not present these two as agreeing or average them; they are different designs answering the same question with opposite implications.

M-2 — Non-inferiority is not superiority, and 6 months is not 3 years. The DCD Heart trial's estimand was *6-month risk-adjusted survival, non-inferiority margin*. Reading it as evidence of long-term durability is a category error in both directions.

M-3 — Chapman ≠ PROTECT. Both are US liver NMP RCTs; the null one used **OrganOx metra**, the positive one used **OCS Liver**. They also differ in primary endpoint definition and donor risk (Chapman median DRI 1.6). Anyone using Chapman to refute PROTECT, or PROTECT to validate metra, is comparing devices, populations and endpoints simultaneously.

M-4 — The two liver labels are not the same population, and TransMedics' is broader. OCS Liver: DCD **≤55 years, ≤30 min WIT**. OrganOx metra: DCD **≤40 years, ≤20 min WIT**. Both cap macrosteatosis at ≤15%. A "both are FDA approved for DCD" comparison silently equates a wider indication with a narrower one.

M-5 — "Approved" carries different evidentiary weight per device. OCS Liver: PMA on a positive 300-patient RCT. VitaSmart: **De Novo** (a different pathway) on a 219-patient RCT. LungFX: **PMA despite missing its performance goal with numerically worse 12-month mortality**. Ranking these by regulatory status alone inverts the evidence.

M-6 — MAUDE is a numerator without a denominator. The failure reports in C-12 are real and specific, but MAUDE is passive-surveillance, is not a rate, and the only denominator on offer is the manufacturer's own "<0.2%" (C-11). Neither a bull nor a bear case can be built on MAUDE counts alone; the Scorpion petition's under-reporting allegation (T-5b) is precisely a claim that the numerator is understated, and it is unresolved at FDA.

M-7 — The company's per-organ revenue is not product revenue. Q2-26: liver ~\$148M + heart ~\$33M + lung ~\$2M = **\$183M**, against **transplant product revenue of \$111M** and total revenue of \$190M. The organ split therefore bundles logistics/service. Any "device ASP" or "per-case device revenue" derived from the organ split will be inflated by roughly the logistics load.

M-8 — Two terminated TransMedics trials are benign; do not build a kill on them.
NCT04194437 (OCS Liver DCD, n=9) — registry whyStopped: *"Enrollment in Liver DCD was stopped as a result of the FDA approval for the OCS Liver System."* **NCT03343535 (EXPAND II Lung, n=46)** — *"Enrollment in EXPAND II was stopped June 24, 2019 as a result of the FDA approval for the EXPAND indication."* Both terminations are administrative consequences of approval, not safety or futility stops.

M-9 — "Utilization rate 89%" is not an outcome. It is the fraction of *perfused* DCD hearts that were transplanted (90/101). It says nothing about survival and cannot be compared to a survival percentage.

M-10 — The ENHANCE Part B control arm is a sponsor device. If the CHOPS IDE amendment is granted, the "static cold storage" comparator will be TransMedics' own investigational active-cooling device rather than conventional phase-change cold storage. Whichever way the trial reads out, the comparator is not the standard of care that clinicians actually use today, and a superiority result would be against CHOPS specifically.

M-11 — DPP ≠ OCS in registry data. UNOS/SRTR cannot reliably identify which perfusion device was used; the SRTR-based study explicitly notes the registry “poorly captures organ storage/transport practices” and that misclassification is possible. “DPP” is a proxy for OCS in the US, not a measurement of it.

5. DATED TECHNICAL CATALYSTS

Date	Event	Confirmation	Existence independently verifiable?
2026-08 (passed)	Heart EXPAND + CAP continued follow-up, primary completion	Date-confirmed (registry)	Yes — NCT05096299 . Results not yet posted as of my pull
2026-10	OCS DCD Heart + CAP continued follow-up, primary completion, n=276	Date-confirmed (registry) , last updated 2025-09-09	Yes — NCT05741723 . This is the single dataset that most directly addresses M-1 / the 3-year durability question
Late Q3 / early Q4 2026 (i.e. now)	FDA decision on the CHOPS IDE amendment enabling ENHANCE Part B control-arm enrollment; lung IDE amendment “shortly thereafter”	Company-guided , from the Q2-26 call	Partially — the CHOPS device is company-disclosed (2026-04-23) but has no separate public FDA docket. Blocking dependency for Part B
Before YE 2026	ENHANCE Part A completion — mgmt: “going to be completed before year-end this year. There’s no doubt about it.”	Company-guided	Trial itself verifiable: NCT07145515 , 655 est., RECRUITING, 8 listed sites
2027-01-30	ENHANCE primary completion (registry)	Date-confirmed (registry)	Yes. Note this precedes the company’s “12 to 18 months from initiation” enrollment framing — the two are not obviously consistent
“Later this year” (2026)	XVIVO PMA submission for Heart Assist Transport (HOPE) following PRESERVE	Company-guided (XVIVO)	Trial and CAP expansion are independently documented
2027-12	DENOVO Lung primary completion; 450 pts (300 OCS / 150 SCS); primary endpoint patient & graft survival + freedom from PGD3 at 72h	Date-confirmed (registry) — but status is NOT_YET_RECRUITING with a 2026-03 stated start and a last update of 2026-01-14 , i.e. the record is ~8 months stale against a start date now 6 months past	NCT07340710 . Flag: a guided start with an unrefreshed registry record is a data point, not a confirmation
2027 (“later in 2027”)	OCS Kidney first clinical experience ; first pre-IDE meeting with FDA held the morning of the Q2-26 call	Company-guided only	No — there is no registered kidney trial and no public IDE. Per the brief’s own rule this is a flag, not a catalyst . It is nonetheless the stated cause of the FY26 margin guidance cut (V-7)

Date	Event	Confirmation	Existence independently verifiable?
Undated, overdue	FDA final response to Citizen Petition FDA-2025-P-0182 (suspend OCS PMA). Interim response posted 2025-09-09; no final response in the docket as of 2026-09-04	Date-confirmed as to filings; outcome and timing unknown	Yes — public docket
2026-06	OCS Liver Perfusion post-approval registry listed completion (primary completion date of 2024-03-22 already passed with no results posted)	Date-confirmed (registry)	Yes — NCT05074160
2026-10-29	Next earnings print	Vendor-derived (yfinance), UNCONFIRMED — no company PR names it. Per the evidence pack, treat print proximity accordingly	—
